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February 23, 2026

Dr. Thomas Keane

Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology

Attention: Request for Information: HHS Health Sector AI RFI

Mary E. Switzer Building

Mail Stop:7033A

330 C Street, S.W., Washington, D.C. 20201

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

Dear Assistant Secretary Keane:

The Healthcare Trust Institute (HTI) appreciates the opportunity to comment on the Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RFI) published by the Department of Health and Human Services (HHS) Assistant Secretary for Technology Policy/Office of the National Coordinator of Health Information Technology (ASTP/ONC) in the Federal Register on December 23, 2025.¹

HTI is an alliance of healthcare organizations committed to promoting and implementing effective protections for health information that engender trust in the healthcare system and allow for the necessary exchange of health data to advance treatments, cures and improved healthcare quality for individuals and populations. HTI members, which include companies and organizations from across the U.S. healthcare economy, agree that uniform national standards for the protection of health information are critical to protect patient data and spur medical innovation.

We appreciate ASTP/ONC's leadership in seeking input on how to accelerate the

¹ 90 Fed. Reg. at 60108 (December 23, 2025).

adoption and responsible use of artificial intelligence (AI) in clinical care. AI tools already support a wide range of clinical and operational functions across the healthcare sector and have brought significant benefits in terms of improved quality, efficiency, access, and outcomes. Their potential to further improve clinician care and the clinical experience is enormous, but realizing this potential will require a policy approach that encourages innovation, supports adoption at scale, and protects patient safety without imposing unnecessary or duplicative regulatory burden.

A National Framework

ASTP/ONC asks about the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care. Currently, the most significant hurdle for healthcare organizations is having to navigate and comply with a myriad of overlapping, and potentially inconsistent, state laws regulating the use of AI and data, which is a key building block for training AI models. Some of these laws are sector-specific, but many are not. In 2025 alone, approximately 1,250 AI bills were introduced at the state level seeking to regulate almost every aspect of the use of AI², and the pace of new state bills on AI is only increasing, with more and more being enacted each year. The number and variations in state privacy laws has also increased sharply over recent years, with many imposing significant new responsibilities and administrative burdens. Compliance with this proliferation of state laws will be extremely challenging and onerous for the vast majority of organizations that do not operate exclusively within one state.

Establishing a national standard or framework for the use of AI and health data in healthcare is critical to ensure regulatory harmonization and avoid undue burden, inefficiencies, and potential safety issues. For these reasons we support federal preemption of state laws regulating AI and data privacy, especially for key provisions where conflicting requirements would become impracticable or overly burdensome. This would not apply to areas that fall under the authority of states to regulate, but only to AI or privacy-specific laws. To the extent that states regulate or provide guidance in areas that fall under the jurisdiction of states (like commercial insurance, data center infrastructure, child safety protections, etc.), they should be encouraged to take a coordinated approach using common models or frameworks to avoid inconsistencies and variability.

Harmonize with Existing Frameworks to Protect Patients

Patient safety and trust are paramount to the acceptance of utilizing AI in clinical care, and appropriate oversight is critical to ensure that the power of AI solutions benefit patients and does not put their safety or wellbeing at risk. However, we believe that this oversight and regulatory protection can largely be achieved through existing regulatory frameworks, which already provide substantial protections for patients.

This is particularly important in the health sector, where various HHS agencies, such as the Food and Drug Administration (FDA), the Centers for Medicare & Medicaid Services (CMS) and the Assistant Secretary for Technology Policy and the Office of the National Coordinator (ASTP/ONC) have already begun to issue regulations and guidance on the

² [National Conference of State Legislatures AI database](#)

use of AI within their areas of jurisdiction. A one-size-fits-all approach to AI regulation, as is being proposed by some states, is destined to fail as it becomes clear that overlaying a totally new framework over existing, often very different regulatory regimes is impractical and, in many cases, infeasible. For example, the FDA has issued multiple guidance documents on AI and machine learning in software as a medical device. Similarly, HHS has issued regulations under Section 1557 of the Affordable Care Act addressing the use of patient care decision support tools in clinical care, and ASTP/ONC has issued transparency and risk management requirements for Predictive Decision Support Interventions that are part of certified health information technology (HIT), such as electronic health records. Any AI regulation should harmonize with these existing requirements as well as with other regulatory requirements applicable to these entities. It is also critical that the agencies within HHS and across other federal departments that may have jurisdiction over activities of the health care sector coordinate their approach to AI regulation so that there is consistency and harmonization of requirements among agencies and across the sector.

For example, health care providers operate within established guardrails, including FDA oversight where applicable, HIPAA and state privacy laws, professional standards of care, accreditation requirements, and medical malpractice frameworks. These mechanisms provide accountability and patient protections regardless of whether AI tools are used. Similarly, health plans are using AI to support care management, quality measurement, utilization management, and administrative automation such as prior authorization and claims processing. These activities are already subject to program integrity requirements, audit processes, consumer protection laws, and oversight mechanisms that can be applied to AI-enabled functions without creating new, duplicative rules.

Wherever possible, HHS should rely on and reinforce these existing guardrails rather than layering on additional AI-specific requirements. Leveraging established oversight promotes consistency, reduces compliance complexity, and allows health care providers and plans to adopt AI responsibly while maintaining patient trust.

Avoiding Overregulation That Could Inhibit Innovation

Overly prescriptive or technology-specific regulation risks inhibiting innovation, discouraging investment, and delaying the translation of promising tools into real-world clinical use. For example, clinicians are increasingly deploying AI-enabled clinical decision support tools to assist with imaging interpretation, pathology review, and early detection of conditions such as sepsis or stroke. These tools augment clinician judgment rather than replace it and are often refined continuously based on clinical feedback and new evidence. Overly rigid requirements could slow iteration and limit the ability of health care providers to tailor AI tools to their patient populations and care settings.

Similarly, AI-powered documentation and workflow tools—such as ambient clinical documentation, chart summarization, and coding assistance—are helping reduce administrative burden and clinician burnout, allowing clinicians to spend more time on

direct patient care. These tools typically operate in the background of clinical workflows, and prescriptive regulation could inadvertently discourage adoption even where patient risk is low.

ASTP/ONC should prioritize flexible, principles-based approaches focused on outcomes, transparency, and accountability, allowing innovation to evolve while maintaining appropriate safeguards.

ASTP/ONC asks if any HHS regulations, policies, or programs could be revisited to augment the ability to develop or use AI in clinical care. There are several potentially deregulatory actions that HHS could take to encourage the development of AI tools in clinical care. These include addressing regulations that make it more difficult and costly to manage data need to train AI models, such as significantly revising proposed updates to the 2024 HIPAA Security Rule and better aligning the 42 C.F.R. pt. 2 (2025) regulations with HIPAA consistent with amendments enacted in the Consolidated Appropriations Act of 2021, Pub. L. No. 116-260. Working together with Congress, HHS should also seek to strengthen HIPAA's preemption framework and explore ways to extend comparable privacy and security standards for personal health information held by entities not currently subject to HIPAA. Greater nationwide consistency in privacy and security protections would both enhance patient trust and provide clearer rules of the road for responsible AI development and utilization.

Aligning Reimbursement to Incentivize Adoption

Reimbursement policy will be a critical driver of AI adoption in clinical care. Even when AI tools demonstrate clear clinical or operational value, health care providers often lack a sustainable financial pathway to adopt and integrate these technologies.

For example, health systems are using AI to support population health management and risk stratification, identifying patients at high risk for hospitalization or care gaps and enabling earlier, more targeted interventions. AI-enabled tools that improve diagnostic accuracy, care coordination, or patient engagement—such as automated outreach, triage, or follow-up—can improve access and outcomes, particularly in underserved communities. Reimbursement policies that recognize the value of AI solutions will facilitate this type of preventive and/or proactive care by making it financially viable for health care providers. This not only benefits patients by improving care, but ultimately reduces costs for the entire healthcare system.

We encourage HHS to explore reimbursement approaches that recognize and reward appropriate use of AI tools, support integration into clinical workflows, and allow flexibility across care settings and specialties.

ASTP/ONC notes that, for non-medical devices, use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security), and asks what role, if any, should HHS play to help address them. While ASTP/ONC does not define "clinical care" in the RFI, it is important that regulation of AI in the health

sector clearly distinguish between treatment or care provided by human beings, including with the help of AI tools, versus consumer interactions with AI chatbots, such as to ask about symptoms and treatments. The latter should without question be regulated, but not as the provision of clinical care by health care professionals. Instead, we recommend that the Federal Trade Commission oversee these types of generative AI tools to ensure that consumers understand their risks and limitations.

The “supply chain” of AI models makes it challenging to determine responsibility/liability in the event of an inaccurate recommendation. This underscores the importance of transparency standards for vendors, and post-deployment standards for developers to ensure ongoing integrity of tools.

Research and Development

ASTP/ONC asks if there are specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care. We appreciate the approach of regulatory sandboxes. Stakeholders from across the continuum including patients, providers, developers, and payers should be included in these efforts.

Conclusion

Accelerating the adoption of AI in clinical care will require a balanced approach that promotes innovation, aligns reimbursement with value, and relies on existing safeguards to protect patients. By avoiding unnecessary regulation, incentivizing responsible use through payment policy, and building on current oversight frameworks, HHS can help ensure that AI advances clinical care in ways that are safe, effective, and scalable across the health care system.

Thank you for your consideration of our comments. Please do not hesitate to contact me at tina@hctrustinst.com or 202-750-1989 if you have any questions.

Sincerely,

A handwritten signature in black ink that reads "Tina O. Grande". The signature is fluid and cursive, with the first name "Tina" being the most prominent part.

Tina O. Grande
President, Healthcare Trust Institute